

THE CHALLENGES OF HEALTH REGULATION IN NIGERIA: THE BIG PICTURE

A Paper by

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Preamble

The topic of discussion, “*The Challenges of Health Regulation in Nigeria: The Big Picture*”, highlights the critical challenges facing health regulation in Nigeria. By choosing this topic, the Institute of Healthcare Management, Nigeria, emphasizes the need for a comprehensive exploration of these challenges. The topic aligns perfectly with the broader conference theme, “*Healthcare Management in Nigeria: Challenges and Prospects*”, and aims to strengthen service delivery, regulation, and training—the sub-theme of this National Conference.

Healthcare service is a packaged commodity delivered by stakeholders within the healthcare system through registered, inspected, approved, and licensed facilities, subject to periodic evaluation by relevant health authorities. The goal is to make healthcare easily accessible to patients whose health is compromised, to prevent disease, or to maintain bodily normalcy. Key questions arise:

- What type of healthcare package is being dispensed?
- At which facility?
- By which stakeholders?
- With what level of skill, expertise, and competence?

The evaluation of these questions through inspection or accreditation is a primary function of health regulatory agencies. These agencies rely on Human

Resources for Health (HRH), defined by the World Health Organization (WHO) as personnel who promote and preserve health, diagnose and treat diseases, and include health management and support staff who ensure the healthcare system functions efficiently (WHO, 2006). In all formal organizations, human resources are the heartbeat of service delivery. This paper focuses on the role of health regulatory agencies in Nigeria, the use of HRH in healthcare facilities, and the challenges they face.

1. Health Regulatory Agencies in Nigeria

Currently, there are thirteen (13) healthcare regulatory agencies in Nigeria:

1. Community Health Practitioners Registration Board of Nigeria (CHPRBN)
2. Dental Technologists Registration Board of Nigeria (DTRBN)
3. Dental Therapists Registration Board of Nigeria (DTRBN)
4. Health Records Officers Registration Board of Nigeria (HRORBN)
5. Institute of Chartered Chemists of Nigeria (ICCON)
6. Institute of Public Analysts of Nigeria (IPAN)
7. Medical and Dental Council of Nigeria (MDCN)
8. Medical Laboratory Science Council of Nigeria (MLSCN)
9. Medical Rehabilitation Therapists Registration Board of Nigeria (MRTRBN)
10. Nursing and Midwifery Council of Nigeria (NMCN)
11. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)
12. Pharmacists Council of Nigeria (PCN)
13. Radiographers Registration Board of Nigeria (RRBN)

These agencies are empowered to make regulations under the various Acts of the Federation of Nigeria (LFN) that establish them, and to define and advance training and practice within their respective professions. They operate as government parastatals under the supervision of the Federal Ministry of Health (FMOH), each governed by a separate Board or Council.

The core functions of these agencies include the registration and licensing of practitioners, inspection of practice premises, accreditation of training institutions, and ensuring Continuing Professional Development (CPD) compliance. For example, the Pharmacists Council of Nigeria (PCN) was established under Decree (now Act) 91 of 1992 (LFN No. 17 of 2004) to regulate pharmacy education and practice.

1.1 Example: Community Health Practitioners Registration Board of Nigeria

The Community Health Practitioners Registration Board of Nigeria (“the Board”) is responsible for:

- a) Determining standards of knowledge and skills for community health practitioners and improving these standards over time;
- b) Establishing and maintaining a register of registered members and publishing lists periodically;
- c) Conducting examinations and awarding certificates or diplomas, with fees prescribed by the Board;
- d) Performing other functions conferred by the Act.

1.2 The Registrar

Each regulatory agency is led by a Registrar, who serves as the Chief Executive Officer (CEO). Responsibilities include administration according to Public Service Rules. Appointment of a Registrar typically involves advertisement, shortlisting, interview, and selection. The Registrar must be a registered and licensed professional in the respective field.

For example, the Medical and Dental Council of Nigeria (MDCN) Act stipulates:

- The Council appoints a qualified medical practitioner or dental surgeon as Registrar.
- The Registrar maintains registers of names, addresses, qualifications, and other particulars of registered practitioners.

Registrars play a pivotal role in maintaining the integrity of professional registration and regulatory oversight.

1.3 Staff of Regulatory Agencies

Regulatory agencies recruit staff as public officers to support the Registrar in discharging regulatory functions. Staff appointments follow Public Service Rules. For instance, the Nursing and Midwifery Council of Nigeria:

- May appoint additional employees as necessary;
- Holds authority to advance, confirm, promote, transfer, discipline, or terminate staff appointments.

Staff are considered public officers, subject to retirement rules, and receive salaries and benefits approved by the Office of the Accountant-General of the Federation (OAGF).

1.3 Governing Boards/Councils

All regulatory agencies operate under Governing Boards or Councils appointed by the Federal Government to serve as policy-making bodies. Membership composition varies by agency.

For example, the Medical Laboratory Science Council of Nigeria (MLSCN) Act, 2003, stipulates:

- Chairman: A qualified Fellow with at least 5 years post-qualification experience, appointed by the President;
- Representatives from the Federal Ministry of Health, private practitioners, and universities;
- Representatives from professional associations and non-governmental organizations with interest in healthcare.

Most Boards follow similar structures, balancing professional expertise with public representation.

Such is the composition of most of the Boards or Councils. The number of the members on such Boards is produced in the following table:

S/	REGULATORY AGENCY	NUMBER OF
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NO		MEMBERS ON THE BOARD
1.	Pharmacists Council of Nigeria (PCN)	
2.	Medical and Dental Council of Nigeria (MDCN)	
3.	Institute of Public Analysts of Nigeria (IPAN)	
4.	Medical Laboratory Scientists Council of Nigeria (MLSCN)	
5.	Nursing and Midwifery Council of Nigeria (NMCN)	
6.	Institute of Chartered Chemists of Nigeria (ICCON)	
7.	Dental Therapists Registration Board of Nigeria (DTRBN)	
8.	Medical Rehabilitation Therapists Registration Board of Nigeria (MRTB)	
9.	Optometrists and Dispensing Opticians registration Board of Nigeria (ODORBN)	
10.	Dental Technologists Registration Board of Nigeria (DRTBN)	
11.	Radiographers Registration Board of Nigeria (RRBN)	
12.	Health Record Officers Registration Board of Nigeria (HRORBN)	
13.	Community Health Practitioners Registration Board of Nigeria (CHPRN)	

1.4 Variations in Board Composition

There is, however, a difference with the Institute of Chartered Chemists of Nigeria (ICCON) and the Institute of Public Analysts of Nigeria (IPAN) regarding the composition of their Boards and the title of their chairmen, who are called

Presidents. For ICCON, the President and Vice President are elected by the Council. The Act provides:

“There shall be for the Institute a President and a Vice President, who shall be Fellows of the Institute, to be elected by the Council and hold office for a term of two years from the date of election.” (Chapter 112, Institute of Chartered Chemists of Nigeria Act)

The same election process applies for IPAN. While the other eleven (11) regulatory agencies have their chairmen appointed by the President of Nigeria on the recommendation of the Honourable Minister of Health, ICCON and IPAN select their leaders through elections.

A common feature among all regulatory agencies is the establishment of Investigating Panels and Disciplinary Tribunals. For instance, the Medical Rehabilitation Therapists (Registration, etc.) Act provides:

“There shall be established a Disciplinary Committee, known as the Medical Rehabilitation Therapists Practitioners Disciplinary Committee (‘the Disciplinary Committee’), charged with the duty of considering and determining any case referred to it by the Panel established under this Act.” (Chapter M9, Medical Rehabilitation Therapists (Registration, Etc.) Act)

Regulatory agencies exist to ensure the proper education and practice of healthcare professionals. Deviations from standards must be corrected to maintain professionalism and protect the public. Issues of professional misconduct or ethics are investigated by the Panel, after which the Disciplinary Tribunal hears cases publicly. The Attorney-General of the Federation appoints a counsel to assist Panels and Tribunals during proceedings.

2.0 Role of Health Regulatory Agencies in Advancing Healthcare Delivery

Emeritus Professor Umaru Shehu, FAS, emphasized the role of professional regulatory bodies:

“The roles of professional regulatory bodies are usually well defined and similar worldwide. They include:

- Setting minimum entry qualifications for students;
- Approving curricula for training;

- Monitoring student training to ensure compliance;
- Certification and re-certification programmes;
- Facilitating Continuing Professional Development (CPD);
- Ensuring compliance with professional ethics and morality.” (CRHP, 2008)

Prof. Dennis Agbonlahor, former Chairman of MLSCN, stated:

“Today's patient care and healthcare practice is multi-professional and multi-disciplinary. Recognizing that all participants contribute equally valid knowledge and expertise is essential for effective teamwork.” (CRHP, 2011)

At the global level, representatives of the World Medical Association (WMA), the International Pharmaceutical Federation (FIP), and the International Council of Nurses (ICN) formed the World Health Professions Alliance (WHPA) in 1999. Dentists joined in 2005 through the World Dental Federation (WDF), collectively representing 25 million healthcare professionals worldwide. The WHPA promotes collaboration among key health professionals, governments, policymakers, and the World Health Organization (WHO) to benefit patients and healthcare systems (CRHP, 2008).

The author attended the First World Health Professions Conference on Regulations in Geneva, Switzerland (May 17–18, 2008), organized by the WHPA. The conference emphasized the regulatory role of agencies in maintaining healthcare systems and the responsibility of governments to fund inspections and accreditations.

2.1 Accreditation and Inspection of Healthcare Facilities

Healthcare training institutions must be accredited to ensure adherence to approved curricula, Benchmark Minimum Academic Standards (BMAS), or course content. Evaluations can occur in Schools of Health Technology, Polytechnics, or Universities. Teams of professional experts conduct accreditation visits, assessing new and ongoing programs.

Accreditation also applies to internship or housemanship centers for fresh graduates, such as medical doctors, pharmacists, and radiographers. Supervising professionals must demonstrate full oversight. For instance, in PCN, a logbook must be signed by a supervising pharmacist with at least five years of registration

and countersigned by the Director of Pharmaceutical Services (DPS) of the State Ministry of Health.

Inspections ensure that healthcare facilities meet minimum standards of practice and protect the public. They may be for licensing new facilities or monitoring registered premises. Inspectors from MLSCN, PCN, and NMCN assess compliance with professional standards, ensuring that fake or unqualified practitioners do not operate. Protocols, SOPs, monographs, and documents such as PCN's *Four-Part Compendium of Minimum Standards of Practice for the Assurance of Pharmaceutical Care in Nigeria* serve as evaluation tools.

Inspections are carried out in teams. While accreditations are periodic, inspections are ongoing. The PCN has Pharmaceutical Inspection Committees (PICs) in States and the FCT to inspect pharmacies and importation firms. Private pharmacists may serve as co-opted inspectors after receiving training and ID cards. However, inspections of pharmaceutical manufacturing facilities are conducted exclusively by public officers. For RRBN, inspections of imaging centers are centrally managed from Lagos due to the specialized equipment and limited number of facilities.

Inspectors act as “policemen” of healthcare premises. At PCN, inspections are corrective, not punitive. Punitive approaches may discourage cooperation from healthcare professionals.

2.2 Continuing Professional Development (CPD) Programmes

Healthcare professionals must maintain high competence to avoid errors and maintain patient confidence. CPD ensures that practitioners adapt to specialized healthcare demands, emerging diseases, drug resistance, and changing environments.

At the National Council on Health (NCH) meeting in Asaba, Delta State (April 2010), participation in CPDs became mandatory for licensing and license renewal. PCN launched the Mandatory Continuing Professional Education (MCPE) in 1998, later renamed Mandatory Continuing Professional Development (MCPD) in 2004. Initially, some modules were overly academic, but collaboration with Partners for Transforming Health Systems (PATHS) improved content and practical relevance.

2.3 The Committee of Registrars of Health Professions (CRHP)

On August 19, 2004, the Honourable Minister of Health, Professor Eytayo Lambo, inaugurated the CRHP to:

- Raise ethical standards in healthcare delivery;
- Set performance targets;
- Facilitate CPD for healthcare professions;
- Educate the public on health matters;
- Harmonize regulatory activities for efficiency;
- Foster collaboration among healthcare professionals;
- Promote unity and mutual respect among health professions in Nigeria.

The Minister stated:

“It is heart-warming that Registrars of all regulatory bodies are coming together under one umbrella. I urge you to use this Committee as a springboard to rejuvenate the team spirit that is fast fading away.” (Lambo, 2004)

Interconnectivity among regulatory agencies ensures coordinated oversight across primary, secondary, and tertiary healthcare facilities. For example, inspectors from PCN and MDCN may encounter each other in hospitals or clinics.

3.0 Challenges of Health Regulation in Nigeria

The key functions of health regulatory agencies include accreditation, inspections, CPD, and maintaining registers of practitioners and practice premises. Investigating Panels and Disciplinary Tribunals uphold ethics, professionalism, and standards. The following are major challenges and proposed solutions:

3.1 Inadequate Numbers of HRH

Insufficient personnel affect inspection frequency, quality, and integrity. Governments must recruit, train, and adequately remunerate health inspectors, ensuring independence from the institutions they monitor.

3.2 Creation/Upgrading of Housemanship or Internship Centres

Graduates require accredited internship placements for skill development. Universities must ensure sufficient placements in recognized centers.

3.3 Poor Logistics Support for Inspectors

Regulatory agencies need adequate funding for vehicles, communication equipment, and other logistics to conduct inspections effectively.

3.4 Lack of Adequate Training and Development for Inspectors

Inspectors require specialized knowledge. Training in Nigeria and abroad ensures technical proficiency and effective oversight.

3.5 Inadequate CPD Programmes

Some agencies lack CPD implementation or offer poorly designed modules. Experienced practitioners should contribute as resource persons. License issuance should depend on CPD compliance and fitness to practice.

3.6 Interference by Professional Associations

Professional associations, as NGOs, should not usurp regulatory authority. Regulatory staff are public officers trained to implement mandates objectively.

3.7 Frequent Dissolutions of Governing Boards/Councils

Frequent early dissolutions disrupt policy continuity and management supervision. Boards should serve full terms unless incompetence is evident.

4.0 Conclusion

Health regulatory agencies play a vital role in Nigeria. Potential mergers of parastatals should not compromise effective regulation and oversight. Government funding, well-trained public officers, and accountability are essential for maintaining healthcare quality in registered and unregistered facilities nationwide.

Thank you for listening, and God bless.

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